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Practice what you profess: Faculty mental health, compassion fatigue, and cultivating a resilience practice

Encouragement for self-compassion and practices for building resilience.

By [Millie Cordaro, PhD](#) Date created: November 27, 2023 5 min read

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As a fellow psychology educator, I put considerable thought into the integrity underpinning my professional and personal responsibilities as a psychologist, and what I model for my students. When I’m standing in my integrity, I am taking care of myself, especially my mental health.

While I'm not perfect, I try my best to practice what I profess. I want to be fully present for my students when they need me, and I know your students need you as well.

As we collectively move past the apogee of the pandemic, a historical traumatic stressor event, faculty mental health is a topical interest. I had the honor of giving an invited talk on faculty mental health and compassion fatigue at the 2023 APA Convention, and I'm delighted to continue the conversation here. It is my professional aspiration to raise awareness and bring to the forefront faculty mental health and compassion fatigue as professional issues that require attention in higher education.

Faculty experience the most common mental health disorders: major depression disorder (MDD) and generalized anxiety disorder (GAD). My research team found the 2-week prevalence rates for MDD and GAD were 66.7% and 86.4%, respectively, among a nationwide sample of faculty planning to leave higher education (Schmiedehaus et al., 2023). Recognizing the ways in which the current educational landscape contributes to increased stress and mental health concerns and emphasizing the importance of faculty mental health and well-being to higher education sets the stage for systemic changes. We can create and offer programming that can potentially prevent the mental health issues that impede our ability to deliver quality education to students as professional development training. I want to be the voice of vulnerability and acknowledge that some psychology teachers are suffering in silence and masking mental health symptoms for fear of appearing weak and incompetent to colleagues and students. This distress may drive some to withdraw and isolate, consequently thwarting opportunities to heal and flourish through social support. With many of us years away from retirement, silent suffering is not a viable way to move through our careers.

Compassion fatigue feels like emotional exhaustion and numbness, psychological overwhelm, deep fatigue, and arduously going through the motions. Compassion fatigue is a type of traumatization comprised of two components: secondary traumatic stress and burnout (Gentry, 2002).

Secondary traumatic stress occurs when we are supporting students whose life challenges are traumatic and shocking, and we are psychologically overwhelmed endeavoring to assist them (Cordaro, 2020). Our students have traumatic narratives involving domestic violence, homelessness, natural disasters, discrimination, sickness, and death of loved ones. As we support student success, we bear witness to their suffering.

The burnout component of compassion fatigue is an occupational hazard, too. Unlike secondary traumatic stress, burnout tends to build gradually and insidiously. With burnout, we may feel the same emotional and physical exhaustion, and we feel less accomplished even as we attain career milestones and achievements. Burnout includes depersonalization, whereby we feel robotic and detached from our job duties. With burnout, we perceive that our work demands outweigh our inner resources (Gentry, 2002). This level of burnout can lead to a felt sense of hopelessness about our careers, with some teachers existentially questioning whether they should leave the profession altogether. Our research demonstrated that compassion fatigue, if left untreated, is a key risk factor for quitting the profession, and leaving behind a once passionate vocation (Cordaro et al., 2023). Paradoxically, the more we care about our students and our jobs, the more susceptible we are to compassion fatigue.

While we are discussing a type of traumatization, compassion fatigue, I want to be trauma-informed and say my next thoughts from a gentle place. One key risk factor for compassion fatigue is our own unresolved trauma, especially unresolved adverse childhood experiences (ACEs; CDC, 2023). First, if you are a psychology teacher with an ACEs history, it is with deep gratitude and pride that I see you as a model of strength and resilience for others. You are a real-life vision of what it means to overcome adversity and demonstrate to students that our early trauma is not our destiny. For those educators that have yet to address this part of your mental health, I hope you feel inspired to be brave and courageous and reach out to a mental health professional to confront those difficult parts to set them free. When we begin to perceive the world as

less dangerous again, we will be less vulnerable to compassion fatigue. We can rewrite that narrative and more fully practice what we profess.

A striking aspect about the empirically supported treatment interventions for resolving trauma and compassion fatigue (e.g., EMDR, CBT, mindfulness, somatic practices; van der Kolk, 2014) is that they can also be used to cultivate a daily mental health routine or resilience practice. While it's important to seek treatment when we are unwell, we should engage our resilience practice when we feel good. We can practice coping in advance, long before the next life challenge hits, or before compassion fatigue sets in.

One antidote to compassion fatigue is practicing self-compassion, an important part of cultivating a mental health practice. Self-compassion means that we are mindfully treating ourselves with kindness and encouragement. Personally, I practice self-compassion through mindfulness and meditation. These are integral parts of my mental health routine and allow me to be present with my students with enough emotional energy for my family as well. One empirically supported self-compassion practice is loving-kindness meditation (Neff, 2023). Ideally, I like to engage myself somatically first by going for a hike in nature, walking, or practicing gentle yoga. Then, I listen to a guided loving-kindness meditation, whereby you direct loving-kindness to someone you care about and eventually you direct your loving-kindness to yourself.

It is my hope that by continuing the conversation around faculty mental health, aspects of personal mental health practices can be applied at the organizational level, allowing us to build a professional resilience practice with our colleagues together. The onus of faculty mental health is a personal and organizational responsibility. Our students learn best when we feel well.

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About the author



Dr. Millie Cordaro is an applied developmental psychologist, therapist, and senior lecturer in the Department of Psychology at Texas State University since 2008. In addition to her teaching vocation, Dr. Cordaro's research interests include the study of traumatic stressor events on generalized anxiety and major depression disorders, faculty mental health, and compassion fatigue and burnout among faculty in academia.

In 2022, Dr. Cordaro completed her 500-hour yoga teacher training. She has a private practice in Austin, Texas, using mindfulness-based cognitive behavioral therapy, meditation, and yoga with clients. She maintains a daily, personal meditation and yoga practice as part of her mental health routine.

You can follow her on [Instagram](https://www.instagram.com/mindful.teaching.psychologist)  (<https://instagram.com/mindful.teaching.psychologist>).

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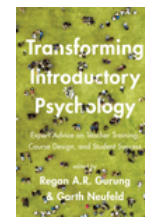
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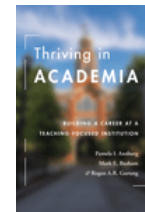
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